



After Hours Clozapine Acquisition Procedure

1. Scope

Site	Operational Area	Applicable to
Armadale Kalamunda Group	All services	Nursing, Medical, Pharmacy

2. Purpose

This document outlines the Armadale Kalamunda Group (AKG) procedure for acquiring clozapine after hours in all clinical areas (including emergency department, general hospital and psychiatric wards).

3. Background

Clozapine is an oral antipsychotic indicated for use in the management of treatment resistant schizophrenia (i.e., in those who have trialled at least two other antipsychotics at optimal doses and for an adequate duration but with poor clinical response or intolerable adverse effects).

Clozapine can only be initiated by a consultant psychiatrist. Medical officers working in the psychiatric unit at AKG can initiate or continue prescribing clozapine on the advice and supervision of a Consultant Psychiatrist if it is medically safe to do so.

4. Procedure for accessing clozapine outside of pharmacy dispensary's opening hours

Where a patient presents to the emergency department (ED) or is admitted to hospital outside of AHS pharmacy dispensary's opening hours of 7:30am- 5:30pm weekdays (or 8am- 12pm weekends and public holidays), clozapine supply must be arranged through the after hours' nurse manager (AHNM), after hours' clinical nurse specialist (AHCNS) or after hours' mental health clinical nurse specialist (AHMHCNS). Clozapine can only be supplied when **all** the following criteria are met:

- Full blood picture (FBP) results are available for the patient and are within the green range or amber range

Green range	white blood cell (WBC) count $> 3.5 \times 10^9/L$ and neutrophil count (NC) greater than $2 \times 10^9/L$	Only supply enough dose(s) until pharmacy reopens the next day
Amber range	WBC $3-3.5 \times 10^9/L$ and/ or NC $1.5-2.0 \times 10^9/L$	Only supply enough dose(s) until pharmacy reopens the next day
Red range	WBC $< 3 \times 10^9/L$ and/ or NC $< 1.5 \times 10^9/L$	Do NOT supply

- The above FBP results must be from a blood test completed within the past 4 days (if in amber range) or from a blood test completed less than 28 days ago (if in green range).

Before referencing this procedure document please ensure that you have the latest version from the [Policy Library](#) information hub page.

Where the blood test was taken outside of the relevant timeframes, a repeat FBP test is to be completed and advice is to be sought from an AKG consultant psychiatrist (or their delegate).

- Patient's usual dose and brand of clozapine (i.e. Clopine® or Clozaril®) has been confirmed. This information can be obtained through the associated clozapine patient monitoring system – ClopineCentral™ for Clopine® or Clozaril Patient Monitoring System (CPMS) for Clozaril®. If this is inaccessible, refer to other sources such as sighting patient's own medication.
- The treating doctor is satisfied that the patient has been compliant with (or has missed less than 48 consecutive hours of) their clozapine treatment immediately prior to this presentation to ED or hospital.
- There are no contraindications (e.g.: suspected myocarditis) for continuation of clozapine
- All clozapine prescribing must be in consultation with an AKG consultant psychiatrist (or their delegate).

Where it is deemed that it is suitable to supply clozapine in the after hours' setting, the AHNM, AHCNS or AHMHCNS must only supply enough dose(s) of the correct brand (Clopine® or Clozaril®) until the hospital pharmacy reopens the following day. Nursing staff should forward a copy of the clozapine order to the hospital pharmacy for further supply (where appropriate) as per usual channels for ordering medications.

Medical officers must be registered with ClopineCentral™ or CPMS (whichever is relevant) in order to prescribe clozapine. Where a patient is admitted to a non– psychiatric ward after hours and is prescribed clozapine (in consultation with the AKG on- call psychiatrist, and in compliance with the above criteria), the patient must be physically reviewed by a consultant psychiatrist (or delegate doctor) within the next 24 hours. The consultant psychiatrist (or delegate doctor) must rechart the clozapine order, if appropriate to continue treatment.

5. Legislative context

The following documents are required to give effect to this procedure:

- *Medicines and Poisons Act 2014*
- Health Department of Western Australia (HDWA) [Medication Review Policy MP 0104/19](#)
- HDWA [Medication Chart Policy M 0078/18](#)
- HDWA [Statewide Medicines Formulary Policy MP 0077/18](#)

6. Supporting information

The following documents support the implementation of this procedure:

- [Medication AKG-POL-0253-13](#)
- [Medication Management AKG-PRO-0255-18](#)

7. Compliance, monitoring and evaluation

Compliance against this procedure will be evaluated and monitored by the Medication Safety Committee.

Monitoring activities include:

- Daily monitoring of the after hours' medication access log.

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 4 – Action 4.14 Safe and secure storage and distribution of medicines
- The health service organisation complies with manufacturers' directions, legislation, and jurisdictional requirements for the:
 - a. Safe and secure storage and distribution of medicines
 - b. Storage of temperature-sensitive medicines and cold chain management
 - c. Disposal of unused, unwanted, or expired medicines

National Standards for Mental Health Services

- Criteria 10.5.6
- Medications are prescribed, stored, transported, administered, and reviewed by authorised persons in a manner consistent with Commonwealth, state/territory legislation and related Acts, regulations, and professional guidelines.

9. References

1. Australian Medicines Handbook 2024 (online). Adelaide: Australian Medicines Handbook Pty Ltd; 2024 February. Available from: <https://amhonline.amh.net.au/>
2. Guidelines for the use of the WA Clozapine Initiation and Titration Chart. WA: Government of Western Australia Department of Health; 2022 February

10. Policy contact (relating to these procedures)

Enquiries relating to this procedure may be directed to:

Title: Pharmacy Head of Department
 Department: Pharmacy
 Email: AHS.

11. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
1.0	October 2018	October 2019	New document
2.0	03/03/2020	03/03/2023	Schedule 3 yearly review
3.0	07/03/2024	07/03/2027	Schedule 3 yearly review

12. Authorisation

This procedure has been authorised and issued by the Executive Director.

Authorised by	Tania Strickland- A/ Director Clinical Services
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